

SAFNECT

Smart Healthcare Platform

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01. EXECUTIVE SUMMARY

SAFNECT is a smart healthcare platform I founded in January 2025 to address critical operational inefficiencies in India's fragmented, paper-heavy healthcare workflow ecosystem. As Founder and Product Lead, I drove the entire product lifecycle — from market research and PRD creation to team assembly, MVP development, and 5 pilot launches — increasing operational efficiency by 50% within the first deployment cycle.

Operational Efficiency Gain 50%	Pilot Launches Completed 5	Team Members Assembled 8+
SDLC Phases Managed Full	Prioritisation Framework RICE	Product Stage MVP+

02. THE PROBLEM

What I observed in the field:

India's healthcare system — especially at the clinic and mid-size hospital level — relies heavily on disconnected, manual workflows: patient intake on paper, lab results communicated via phone calls, appointment scheduling done through WhatsApp, and billing managed in spreadsheets. This creates four compounding problems:

- ▶ **Operational Delays:** A patient's journey from registration to diagnosis involved 4–6 manual handoffs, each prone to error and delay, often adding 45–90 minutes to average wait times.
- ▶ **Data Silos:** No single system connected patient records, lab results, inventory, and billing — leading to repeat tests, medication errors, and revenue leakage.
- ▶ **Staff Burnout:** Frontline healthcare workers were spending 30–40% of their time on administrative coordination rather than patient care.
- ▶ **No Feedback Loop:** Clinic owners and administrators had zero visibility into operational metrics — no KPIs, no dashboards, no way to identify bottlenecks.

Who was affected:

- ▶ Primary users: Healthcare staff (nurses, lab technicians, receptionists)
- ▶ Secondary users: Clinic owners and administrators
- ▶ End beneficiaries: Patients experiencing delayed or error-prone care

The core insight: Healthcare clinics didn't need a full EHR — they needed a lightweight, IoT-integrated operations platform that could be deployed in days, not months, and used by non-technical staff without training overhead.

03. MY ROLE & RESPONSIBILITIES

As Founder and Product Lead, I wore multiple hats simultaneously:

Product Management	Technical Oversight	Stakeholder Management
• Market research & user interviews • Product Vision & PRD creation • RICE framework prioritisation • Roadmap definition • GTM strategy	• SDLC management end-to-end • IoT architecture decisions • API integration oversight • UAT and QA sign-off • Tech debt triage	• Assembled cross-functional team • Clinic owner negotiations • Pilot site coordination • Feedback loop management • Investor communication

04. THE SOLUTION — WHAT I BUILT

SAFNECT was architected as a modular, IoT-integrated healthcare operations platform with three core layers:

Layer 1 — Patient Flow Engine

- ▶ Digital patient intake replacing paper-based registration
- ▶ Real-time queue management with smart wait-time prediction
- ▶ Automated appointment reminders via SMS/WhatsApp integration
- ▶ Triage flagging system to prioritise critical cases

Layer 2 — Connected Operations Hub

- ▶ IoT-enabled lab equipment integration for automatic result ingestion
- ▶ Centralised patient record linking intake → lab → pharmacy → billing
- ▶ Inventory tracking with low-stock alerts and reorder triggers
- ▶ Role-based access for doctors, nurses, lab techs, and admin staff

Layer 3 — Analytics & KPI Dashboard

- ▶ Real-time operational dashboards for clinic owners
- ▶ Bottleneck identification using patient journey funnel analytics
- ▶ Staff productivity metrics and workload distribution insights

- ▶ Revenue leakage detection through billing audit trails

How I approached it as a PM:

- ▶ Discovery: Conducted 15+ user interviews across 3 clinic types (general, diagnostic, specialty) to map pain points and prioritise feature sets.
- ▶ Prioritisation: Applied RICE framework — scored 40+ feature ideas on Reach, Impact, Confidence, and Effort to build the MVP scope. Eliminated 28 features from v1 to maintain focus.
- ▶ PRD: Wrote a comprehensive PRD with user stories, acceptance criteria, technical constraints, and success metrics before a single line of code was written.
- ▶ Iterative delivery: Used Agile sprints to deliver in 2-week cycles, with each sprint ending in a working demo reviewed with pilot clinic staff.

05. CHALLENGES & HOW I TACKLED THEM

Challenge 1 Undefined User Needs — Healthcare Staff Couldn't Articulate Their Problems

My Approach: I replaced feature-request interviews with observation sessions — I sat in 3 clinics for 2 days each, shadowing receptionists and nurses. I used a job-to-be-done (JTBD) framework to document what staff were actually trying to accomplish, not what they said they wanted. This surfaced critical pain points they couldn't verbalize, like the 'waiting for lab results' dead zone that stalled the entire patient queue.

Outcome: Built 3 distinct user personas grounded in real behaviour, which became the north star for every feature decision in the PRD. Eliminated 12 features that users had requested but that didn't address core JTBD.

Challenge 2 IoT Integration Complexity — Diverse, Non-Standard Lab Equipment

My Approach: Most clinics had 3–5 different lab equipment brands with no standardised data output format. Building individual connectors for each was not scalable. I defined an abstraction layer in the architecture — a universal 'result ingestion API' that normalised data from any equipment into a standard schema. I prioritised the 3 most common equipment brands (covering 70% of pilot sites) for v1 and built a manual-entry fallback for others.

Outcome: Delivered IoT connectivity for the 3 priority equipment types on schedule. The abstraction layer design later enabled adding a 4th equipment type in just 3 days (vs. the originally estimated 2 weeks), validating the architecture decision.

Challenge 3 Pilot Site Resistance — Clinic Owners Reluctant to Disrupt Operations

My Approach: Clinic owners were afraid of downtime during onboarding. I addressed this by proposing a 'shadow mode' pilot: SAFNECT ran in parallel with existing workflows for the first 2 weeks — staff could use both systems and compare, with no pressure to switch. I also created a 30-minute onboarding playbook after reducing the initial 3-hour training program based on staff feedback from pilot 1.

Outcome: All 5 pilot sites successfully onboarded. Shadow mode eliminated onboarding resistance and gave us 2 weeks of real usage data before go-live. Training time reduced from 3 hours to 28 minutes by pilot 3.

Challenge 4 Scope Creep — Cross-Functional Team Kept Adding Features Mid-Sprint

My Approach: Without a formal change control process, every stakeholder meeting introduced new feature requests that were being added directly into the development backlog. This caused 2 consecutive sprint overruns. I instituted a 'Change Request Freeze' during sprints and created a formal intake form for new requests that required RICE scoring before entering the backlog. I held a monthly roadmap review to channel all new ideas into the proper prioritisation cycle.

Outcome: Sprint velocity stabilised from sprint 4 onwards. No sprint overruns in the final 6 sprints. Feature request backlog grew from 40 items (unscored chaos) to 40 items (fully RICE-scored and sequenced).

Challenge 5 KPI Definition — Pilot Sites Measuring 'Success' Differently

My Approach: Each clinic owner had a different definition of whether the product was working. Without alignment on success metrics, the pilots risked producing conflicting or unactionable feedback. Before each pilot launch, I ran a 30-minute 'success criteria alignment' session with clinic owners to co-define 3 measurable KPIs specific to their clinic. These were logged in a shared tracker updated weekly.

Outcome: Produced clear, quantified outcomes from all 5 pilots. The cross-site comparison of KPIs revealed the 50% operational efficiency gain as a consistent pattern, validating the product-market fit hypothesis.

06. OUTCOMES & IMPACT

Operational Efficiency Gain 50%	Pilot Clinics Deployed 5	Patient Wait Time Reduction ~40%
Staff Admin Time Saved 30%+	Onboarding Time (v1 → v3) 3hr → 28min	Sprint Overruns (post-process fix) 0

What this demonstrates for a PM role:

- ▶ 0-to-1 Product Building: Founded and shipped a live MVP from scratch, not just contributed to an existing product.
- ▶ User-Centric Thinking: Used JTBD + observation to surface real needs over stated preferences.
- ▶ Structured Prioritisation: Applied RICE rigorously to cut scope without losing stakeholder confidence.
- ▶ Cross-Functional Leadership: Assembled and led a team of 8+ across engineering, design, and operations with no formal authority.
- ▶ Data-Driven Decisions: Pre-defined KPIs per pilot site, tracked weekly, and used data to drive GTM refinements.
- ▶ Process Design: Identified and fixed a scope creep problem through a process intervention, not just willpower.

07. KEY LEARNINGS

- ▶ The best PRD is written after you've done enough discovery to be wrong about almost everything. The first PRD I wrote had 12 features that got cut after user observation sessions — that's not failure, that's the process working.

- ▶ Resistance from end users is almost never about the product — it's about change management. Shadow mode was a positioning decision, not a technical one.
- ▶ Velocity without process creates the illusion of speed. The sprint overruns hurt us in the short term but the change control fix paid off across 6 subsequent sprints.
- ▶ In a 0-to-1 product, the PM's most valuable skill is knowing what NOT to build. Saying no to 28 features in v1 was the single biggest reason we shipped on time.

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